



CDC Updates Recommendations for Travelers’ Diarrhea

Diarrhea Severity

- **Mild Diarrhea:** Tolerable, not distressing, does not interfere with planned activities.
- **Moderate Diarrhea:** Distressing or interferes with planned activities.
- **Severe Diarrhea:** Incapacitating or completely prevents planned activities.
- All **dysentery** is considered severe.

Treatment Options by Diarrhea Severity

Bismuth subsalicylate

Generally not recommended for children <12 years of age.
Not recommended during pregnancy.

Loperamide

Not recommended for management of infectious diarrhea in peds patients.

Mild Diarrhea

- Antibiotic treatment is not recommended.
- Consider treatment with bismuth subsalicylate or loperamide.

Moderate Diarrhea

- Antibiotics can be used for treatment.
 - Azithromycin
 - Fluoroquinolones
 - Rifaximin (for moderate, noninvasive diarrhea)
- Anti-motility drugs
 - Consider loperamide for use as monotherapy or as adjunctive therapy.

Severe Diarrhea

- Antibiotic treatment is advised (single-dose regimens may be used)
 - Azithromycin is preferred.
 - Fluoroquinolones or rifaximin can be used for severe, non-dysenteric diarrhea.
- Anti-motility drugs
 - Consider loperamide for use as adjunctive therapy.
 - Not recommended as monotherapy for patients with bloody diarrhea, or diarrhea and fever.



Note. From Zithromax (Azithromycin) tablets [Photograph], by Anonymous1941, 2019, Wikimedia ([https://commons.wikimedia.org/wiki/File:Zithromax_\(Azithromycin\)_tablets.jpg](https://commons.wikimedia.org/wiki/File:Zithromax_(Azithromycin)_tablets.jpg))

Antibiotics Dosing Guidelines

The [CDC Yellow Book](#) provides a dosing schedule for antibiotics used to treat travelers’ diarrhea (age 18+).

Acute Diarrhea Antibiotic Treatment Recommendations		
Antibiotic	Dose	Duration
Azithromycin	1,000 mg	Single or divided dose
Azithromycin	500 mg QD	3 days
Ciprofloxacin	750 mg	Single dose
Ciprofloxacin	500 mg BID	3 days
Levofloxacin	500 mg QD	1-3 days
Ofloxacin	400 mg BID	1-3 days
Rifamycin SV	388 mg BID	3 days
Rifaximin	200 mg TID	3 days

Travelers’ Diarrhea Caused by Protozoa

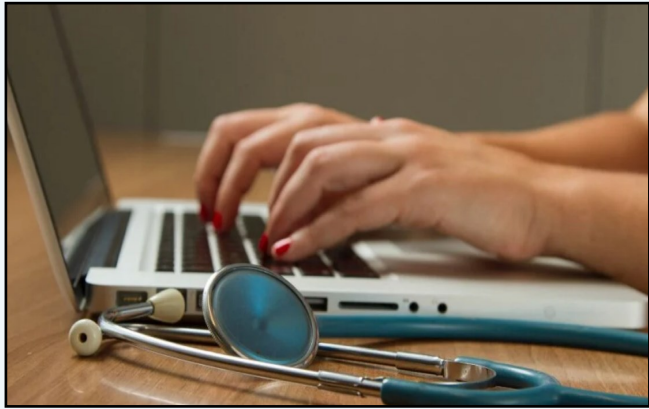
The most common parasitic cause of TD is Giardia duodenalis, and treatment options include metronidazole, nitazoxanide, and tinidazole (see Sec. 5, Part 3, Ch.12, [Giardiasis](#)). Amebiasis (see Sec. 5, Part 3, Ch. 1, [Amebiasis](#)) should be treated with metronidazole or tinidazole, then treated with a luminal agent (e.g., iodoquinol or paromomycin). Although cryptosporidiosis is usually a self-limited illness in immunocompetent people, clinicians can consider nitazoxanide as a treatment option.

Pediatric Travelers’ Diarrhea Treatments

The main treatment for travelers’ diarrhea in children is oral rehydration solution (ORS). ORS should be mixed as directed with boiled or treated water (generally 1 liter). Due to their saltiness, most find ORS generally difficult to drink. In mild cases, rehydration can be achieved with any preferred liquid, including sports drinks. Overly sweet drinks can cause osmotic diarrhea.

Empiric antibiotic treatment in children should be avoided.

Introducing the GPC Mailbag!



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This year, we’d like to expand our scope to include your suggestions. We’re proud to announce the GPC Mailbag, where you can request topics for discussion, or even specific questions, that you’d like to see in a future newsletter. You can reach us at GPCMailbag@kp.org. We look forward to hearing from you!

References

Anonymous1941. (2019). *Zithromax (Azithromycin) tablets* [Photograph]. Wikimedia. [https://commons.wikimedia.org/wiki/File:Zithromax_\(Azithromycin\)_tablets.jpg](https://commons.wikimedia.org/wiki/File:Zithromax_(Azithromycin)_tablets.jpg)

Connor, B. (2024). Travelers’ Diarrhea. Centers for Disease Control and Prevention. <https://wwwnc.cdc.gov/travel/yellowbook/2024/preparing/travelers-diarrhea>

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